National Implementation Guidelines

for Long Acting Injectable Cabotegravir (CAB-LA)

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Foreword

The approval and implementation of the National Policy on HIV Pre-Exposure Prophylaxis (PrEP) and Test and Treat (T&T) in 2016, as well as the introduction of antiretroviral (ARV) treatment as oral PrEP, mark a significant milestone in the fight against HIV in South Africa. The expansion of HIV prevention options through long-acting injectable Cabotegravir further enhances the choices available to individuals at risk of HIV, providing a diverse range of HIV prevention methods.

The goals outlined in the National Strategic Plan (NSP) for 2023-2028 underscore the commitment to making PrEP widely available to those who need it, incorporating long-acting PrEP products, and ensuring its availability at all public primary health care facilities. To this end the availability of PrEP as part of combination HIV prevention in the 3 5000 public Primary Health Care facilities in South Africa, has indeed been a game changer in HIV prevention, making PrEP more accessible to all persons who need them. The introduction of the long acting injectable Cabotegravir will provide an additional HIV prevention option.

We are encouraged that since 2016, over 1.2 million South Africans from diverse backgrounds have chosen to use PrEP as an HIV prevention option. This is a testament to their commitment to their health and to achieve the broader goal of ending AIDS as a public health threat by 2030. The extensive reach of HIV prevention services including PrEP across 3,500 public health facilities, university campus health clinics, and special clinics further highlights the importance expanding accessibility and availability of this vital intervention.

The insights already gained from both PrEP users and healthcare facilities has been invaluable in shaping and improving the delivery of HIV PrEP, which is essential for evidence-based healthcare. The introduction of long-acting injectable Cabotegravir together with oral-PrEP will provide further insights on the delivery of an array of HIV PrEP interventions. The inclusion of long-acting injectable Cabotegravir as an additional HIV prevention intervention is a significant step forward in reducing new HIV infections and reflects a commitment to the health and well-being of the South African population.

I take this opportunity to acknowledge the invaluable role that healthcare professionals, including nurses, doctors, counsellors, health promoters, and researchers, have played in offering quality HIV prevention services. Their dedication and expertise are critical in ensuring the success of these guidelines and in delivering high-quality healthcare services to the South African public. Healthcare workers and researchers will find these guidelines invaluable in their quest to continue to offer quality HIV prevention services to the South African public.

Dr SSS Buthelezi
Director General: Health
Date: 07/12/2023

TABLE OF CONTENTS

Table of Contents

LIST OF BOXES

LIST OF JOB AIDS

LIST OF ACRONYMS & ABBREVIATIONS

List of acronyms and abbreviations

AIDS	Acquired immunodeficiency syndrome
AHI	Acute HIV infection
ALT	Alanine transaminase
ART	Antiretroviral therapy
ARV	Antiretroviral
CAB	Cabotegravir
CAB-LA	Long-acting injectable cabotegravir
DOH	Department of Health
FTC	Emtricitabine
GBV	Gender-based violence
HBsAg	Hepatitis B surface antigen
HBV	Hepatitis B virus
HCV	Hepatitis C virus
HIV	Human immunodeficiency virus
HTS	HIV testing services
INSTI	Integrase strand transfer inhibitor
IPV	Intimate partner violence
ISR	Injection-site reaction
NSAID	Non-steroidal anti-inflammatory drug
NSP	National Strategic Plan
PEP	Post-exposure prophylaxis
PrEP	Pre-exposure prophylaxis
SAHPRA	South African Health Products Regulatory Authority
SRH	Sexual and reproductive health
STI	Sexually transmitted infection
T&T	Test and treat
TasP	Treatment as prevention
TB	Tuberculosis
TDF	Tenofovir disoproxil fumarate
WHO	World Health Organization

PART A: INTRODUCTION

Background

In 2015, the World Health Organization (WHO) recommended that HIV-negative people who are at a substantial risk of acquiring a human immunodeficiency virus (HIV) infection should be offered daily oral HIV pre-exposure prophylaxis (oral PrEP) as part of a combined HIV prevention strategy(1). Over time, there has been growing recognition that people need a range of HIV prevention options to choose from in accordance with their circumstances, needs and preferences. To this end, in addition to oral PrEP, the WHO recommended that both the dapivirine vaginal ring (the Ring) and long-acting injectable cabotegravir (CAB-LA) should be offered as additional HIV prevention options, together with other combination prevention interventions, in order to increase access and uptake of comprehensive HIV prevention approaches(2). The WHO has developed evidence-informed guidelines to support the provision of biomedical interventions and expanded choice. These guidelines reference the two other approved biomedical HIV prevention methods (oral PrEP(5) and the Ring (6)), and provide detailed implementation guidance for CAB-LA.

WHO recommendation on CAB-LA

"Long-acting injectable cabotegravir may be offered as an additional prevention choice for people at substantial risk of HIV infection, as part of combination prevention approaches (conditional recommendation; moderate certainty of evidence)."

WHO, 2022(3)

HIV biomedical prevention in South Africa

South Africa introduced oral PrEP as part of its expanded HIV prevention strategy in 2016(4). Since then, the provision of oral PrEP has expanded from specific targeted populations and sites (sex workers, men who have sex with men (MSM), adolescent girls and young women (AGYW), transgender persons (TG), and people who inject drugs (PWID)) to include all persons who are at substantial risk of HIV or who request PrEP at public sector primary health care clinics. In 2021, the South African national oral PrEP implementation guidelines(5) were revised to ensure alignment with the latest global evidence and WHO guidelines(2).

There has been recognition of the necessity to expand HIV prevention options in South Africa. Building on the experience of oral PrEP implementation, the South African Health Products Regulatory Authority (SAHPRA) approved the dapivirine vaginal ring in March 2022. In support of this, national dapivirine vaginal ring guidelines were developed and approved in December 2022(6).

As part of this move to expanded choice, SAHPRA approved CAB-LA in November 2022. It is currently being provided at select implementation science sites, followed by a phased approach to expanded provision, depending on cost and supply chain issues. These guidelines have been developed to support the introduction of CAB-LA in South Africa.

Key features of CAB-LA

This section provides an at-a-glance overview of the key features of CAB-LA, with additional details in Part B of the guidelines.

Overview of the key features of CAB-LA

Item	Description
Registered name	APRETUDE (hereafter referred to as CAB-LA)
Description	**Both a tablet and an injectable form:** • Tablet 30 mg film coated (cabotegravir (as cabotegravir sodium)) • Injectable 600 mg/3 mL: Prolonged release suspension for injection (cabotegravir 200 mg/mL) - CAB-LA contains 600 mg of cabotegravir extended release injectable suspension. It is an intramuscular injection injected into the gluteal muscle.*
Formulation	**Long-acting cabotegravir injectable 600 mg/3 mL:** • Each single-dose 3 mL vial contains 600 mg cabotegravir • Contains sugar (mannitol 35.0 mg/mL) • A white to light pink, free-flowing suspension
Regulatory approval	• Approved by the United States Food and Drug Administration for use in the USA (December 2021) • Approved by SAHPRA for use in South Africa (30 November 2022)
Mechanism of action	• Cabotegravir belongs to a class of antiretrovirals (ARV) called integrase strand transfer inhibitors (INSTI) that reduce the ability of HIV to replicate itself inside a healthy cell. • CAB-LA delivers cabotegravir systemically, so that the drug is absorbed throughout the body and provides protection for HIV throughout the body, irrespective of the entry point.
Schedule classification	Classified as a Schedule 4 drug (only appropriately trained healthcare providers are authorised to assess, diagnose, prescribe and dispense it – it has the same classification as ARVs, tenofovir disoproxil fumarate (TDF)/emtricitabine (FTC) used for oral PrEP and the dapivirine vaginal ring).
Frequency of injection	Four weeks (one month) after the first injection, then every eight weeks (two months) thereafter. [The first injection on initiation; the second injection in four weeks (one month); the third injection in eight weeks (two months); and thereafter repeated every eight weeks (two months).]
Lead-in period	Current evidence shows it takes an estimated seven days for drug concentrations to reach levels anticipated for CAB-LA to be maximally effective after the initiation injection, clients should be counselled on using another HIV prevention strategy in the first seven days after initiation.
Shelf-life and storage	• Store at 2°C to 25°C (36°F to 77°F). Exposure up to 30°C (86°F) permitted. • Do not freeze. • Prior to administration: - If the pack has been stored in the refrigerator, the vial should be brought to room temperature using the palm of the hand prior to administration – not to exceed 30°C (86°F). - Once CAB-LA has been drawn into the syringe, the medication can remain in the syringe for up to two hours before injecting it. The filled syringes should not be placed in the refrigerator. If the medicine remains in the syringe for more than two hours, the filled syringe and needle must be discarded.
Indication	• For individuals who perceive themselves to be at risk of acquiring HIV and want to take PrEP. • Adults and adolescents who are HIV-negative and weigh ≥35 kg.
Mechanism for administration	• For injection: Single-dose vial of 600 mg/ 3 mL injection into the gluteal muscle in the buttocks.

Item	Description
Duration of effectiveness	• CAB-LA is effective seven days after the first injection. • Thereafter, it is highly effective as long as clients have their follow-up injections as per schedule to ensure that there is sufficient cabotegravir in the system to prevent HIV. **Note:** If a client misses a scheduled injection or discontinues CAB-LA, a concentration of the medication remains in the body. The waning concentration of cabotegravir (CAB) in the body remaining after the injections have ceased is referred to as the pharmacokinetic tail (hereafter referred to as "tail period"), which could persist for an approximate period of one year after the last injection. The concentration of CAB during this period is subtherapeutic and therefore too low to protect against HIV infection, but may provide enough selective drug pressure for CAB resistance to emerge.
Efficacy	• In clinical trials assessing the effectiveness of CAB-LA, CAB-LA was not compared to a placebo, but to daily TDF/FTC oral PrEP. • While both oral PrEP and CAB-LA were found to be highly effective, CAB-LA provided even greater protection to HIV acquisition compared to oral PrEP (HPTN 083 Phase IIb and HPTN 084 Phase IIb/III "efficacy studies" ([7–9]). • The greater efficacy of CAB-LA (over 96%) compared to oral PrEP can be attributed to improved adherence.
Safety	• The HPTN 083 and HPTN 084 trials showed CAB-LA to be generally safe and well tolerated.
Side effects	• Side effects are generally mild to moderate and decrease over time. • The most common side effects include headache, nausea, diarrhoea, fatigue (similar to oral PrEP) and injection-site reactions (ISRs). - ISR is the most common side effect and lessens over time.
Benefits to CAB-LA users	• Convenient; requires two monthly injection appointments after the first month (does not require monthly insertion or daily pill taking). • Highly effective in decreasing risk for HIV. • Discrete – allows user to maintain privacy.
HIV, STI pregnancy prevention	• Does not protect against sexually transmitted infections (STIs) or pregnancy – requires simultaneous correct and consistent condom use, and the use of reliable contraceptives.
Drug interactions	**CAB-LA should not be used by people on:** • Tuberculosis (TB) treatment that includes rifampicin or TB-preventative treatment regimens that include rifabutin or rifapentine. • Some anticonvulsants (carbamazepine, oxcarbazepine, phenobarbital and phenytoin). (See Section 11.)

Guiding principles underpinning service delivery

As with oral PrEP and the Ring, the provision of CAB-LA is underpinned by several key guiding principles. These include the following:

Integration: All PrEP methods need to be provided as part of HIV combination prevention (Box 1) and comprehensive sexual and reproductive health (SRH) services (Box 2).

Choice and informed decision: Provide clients with information about all available HIV prevention options to enable them to make an informed choice (Job Aids 5,6,8).

Access: It is important that people wanting to prevent HIV have full access to available HIV prevention options, alongside SRH services.

Quality of care: Services need to be provided within the framework of quality health care. This embraces the Batho Pele principles(10) and the Ideal Clinic quality criteria(11).

A rights-based approach: All services should be provided in an environment that respects clients' rights, including those relating to SRH and HIV.

Box 1: Combination prevention